



TEAM NAME: **NOTHING**

COLLEGE NAME: **Sri Venkateswara College Of Engineering**

PROBLEM STATEMENT:

The life-saving 'Golden Hour' is being fatally lost to a system of system of blind hospital routing, urban traffic gridlock, and manual ER paperwork



PROBLEM STATEMENT

In densely populated Indian cities like Chennai, emergency medical situations are often chaotic and uncoordinated. Delays in accessing critical care frequently lead to preventable loss of life, increased complications, and heightened strain on emergency services.

Lack of Centralized Data



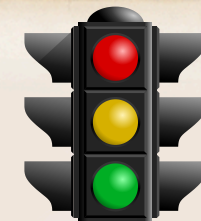
There is no single platform to find hospitals with real-time ICU/ward availability, specialized equipment, and affordable care options.

Delayed Hospital Access



Locating the nearest medically suitable hospital is severely time-consuming during a high-stress crisis.

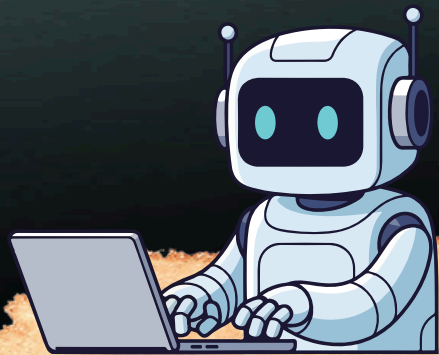
Traffic & Road Clearance



Ambulances suffer fatal delays due to severe urban congestion and a lack of automated coordination with police for rapid road clearance.

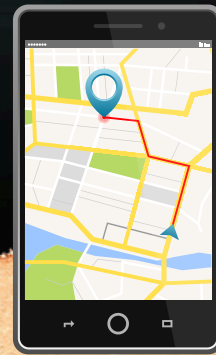
SOLUTION

An AI-driven emergency response and hospital finder platform aims to eliminate fatal delays during the critical "Golden Hour" of a medical crisis. It actively manages the entire emergency logistical chain, unlike traditional SOS apps that only share locations.



Symptom Input & AI Triage

local AI engine maps data to standard clinical triage severity categories.
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Telemetry & Infrastructure Sync

System aggregates precise GPS telemetry, simulated traffic conditions, and real-time hospital bed capacities.



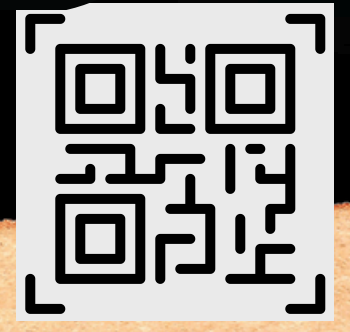
Algorithmic Hospital Matching

Scoring algorithm calculates the optimal destination using distance, medical specialization, beds, and patient affordability.



Transit & Volunteer Alert

App triggers local volunteer alerts and displays context-specific first-aid protocols during ambulance transit.



Decentralized Hospital Intake

Offline QR code serializes critical patient medical history for instant hospital triage ingestion.

FLOW DIAGRAM

Phase 1: Incident & Rapid Triage

1. Rapid AI Triage:

User inputs symptoms, local AI instantly categorizes severity using clinical protocols.

Phase 2: Telemetry & Algorithmic Matching

2. Smart Matching:

System evaluates live traffic, distance, and bed telemetry to calculate the optimal hospital.

Phase 3: Automated Coordination & Dispatch

3. Green Corridor:

App locks the route and transmits a live GPS payload to police for automated intersection clearance.

Phase 4: Transit & Golden Minute Intervention

4. Golden Minute Transit:

System alerts nearby medical volunteers and displays dynamic first-aid instructions.

Phase 5: Decentralized Hospital Intake

5. Instant Intake:

Hospital staff scan the patient's offline QR code for immediate medical history decryption.



TECHNICAL REQUIREMENTS

USER ACCESS



Public User Mobile App
Ambulance Driver App
Hospital Admin Dashboard

MOBILE APPLICATION LAYER



Emergency Mode Controller
Symptom Input Module
Hospital Finder Interface
Navigation & Map Interface

AI PROCESSING LAYER



AI Triage Engine
Hospital Recommendation Algorithm
Traffic Analysis Module
Emergency Alert Generator

DATA PROCESSING LAYER



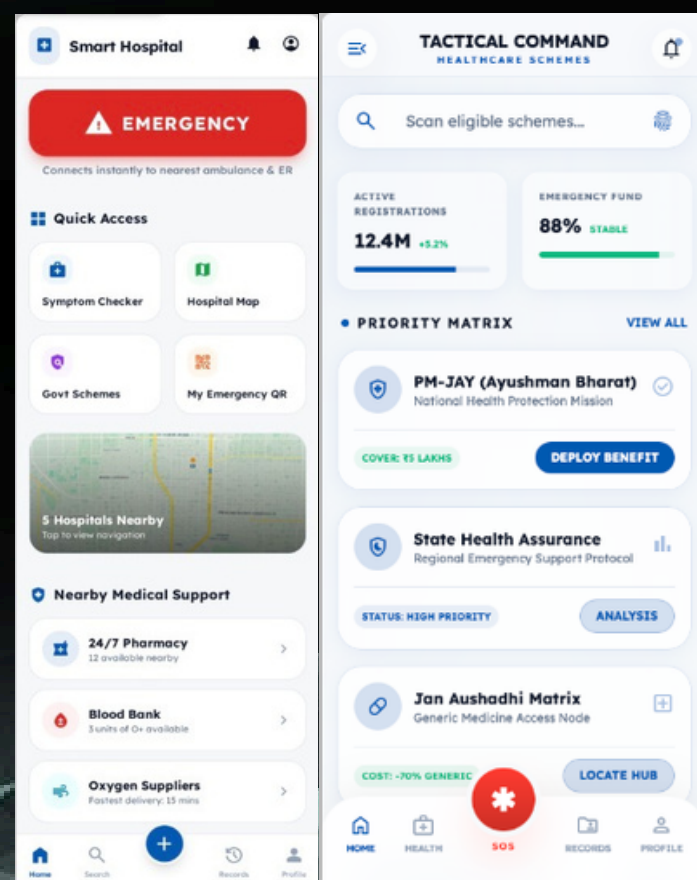
Hospital Database
Bed Availability Registry
Medical Equipment Database
Government Health Scheme Data

External Integration Layer



Google Map Navigation
Emergency Communication System
Traffic Police / Ambulance Integration
Hospital Notification System

APP DEMO



TECH STACK



NOVELTY / USE CASE

Offline-First Edge AI

Processes AllIMS Triage Protocols entirely on-device. Zero downtime during network blackouts.

Smart Routing Matrix

Calculates destination score using real-time traffic, ICU capacity, specialty, and ABHA/PMJAY eligibility—not just proximity.

The Bystander

AI instantly maps visual symptoms to the optimal hospital, dispatches help, and provides offline first-aid.

The Responder

Auto-plots a "Green Corridor" for immediate traffic clearance and broadcasts live ETAs to the destination ER.

Novelty

Use Case

Automated Green Corridors

Transmits live telemetry (GPS+ETA) directly to traffic authorities for intersection clearance.

Decentralized ER Intake:

Uses AES-encrypted Offline QR Codes to bypass 15-minute manual paper delays upon arrival.

The ER Staff

Scans offline QR codes to instantly ingest medical history and ABHA data, bypassing fatal manual paperwork delays.

The Medical Volunteer

Triggers system-override alerts to nearby medical professionals for immediate, on-foot patient stabilization before the ambulance arrives.

BUSINESS MODEL / TARGET MARKET

B2G (Government)

Traffic and health departments requiring automated "Green Corridor" infrastructure.

B2B (Hospital Networks)

Public and private Emergency Rooms seeking to eliminate 15-minute manual intake bottlenecks and optimize ICU bed turnaround.

B2G Infrastructure Licensing

Annual subscription contracts for smart city traffic clearance dashboards.

B2B SaaS API:

Tiered fees for hospitals integrating our offline QR intake into their EMRs.

Target Market

B2B (Transport):

Ambulance fleets needing centralized AI dispatch and route synchronization.

B2C (The Citizens)

The 500+ million Indian citizens enrolled in PMJAY/ABHA who need instant, panic-free, and scheme-verified medical routing.

Business & Revenue Model

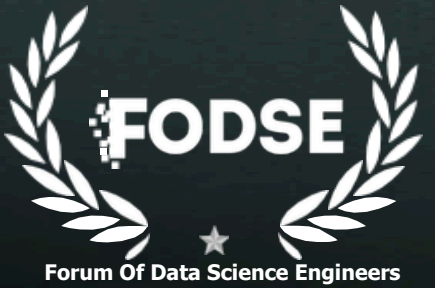
B2C Freemium:

100% free citizen app to drive mass adoption and build the live telemetry network.

Data Analytics

Monetizing anonymized emergency heatmaps to help city planners optimize road infrastructure.

TEAM DETAILS



ROLE	NAME	STREAM/DEPT	YEAR	COLLEGE
Leader	Nanda Kishore Sreejith	Computer Science and engineering	2nd Year	Sri Venkateswara College of Engineering
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Member	Keerthivasan P	Computer Science and engineering	2nd Year	Sri Venkateswara College of Engineering
Member	Keerthan Kumar S	Computer Science and engineering	2nd Year	Sri Venkateswara College of Engineering
Member	Mithun K S	Computer Science and engineering	2nd Year	Sri Venkateswara College of Engineering